

إسم المريض ربيعاً : سحر عبد الرحمن درويش علي

المحافظة : القاهرة الحي / المدينة : المطريه

رقم البطاقة : 26702091400548 رقم التليفون : 01212458404 تاريخ تسجيل الإستمارة : 10-10-2026

العمر(تاريخ الميلاد) : 09-02-1967 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

**قسم العلاج :
الاورام**

خطة العلاج : أبحاث أورام اشعه مقطعيه علي البطن والصدر والحوض بالصيغه [11127]

التعليق على التشخيص : -----

الاسم: د/محمد كيلاي
 الاسم: د/ندي حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات المرفقة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جھة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: سحر عبد الرحمن درويش علي National ID: 26702091400548	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

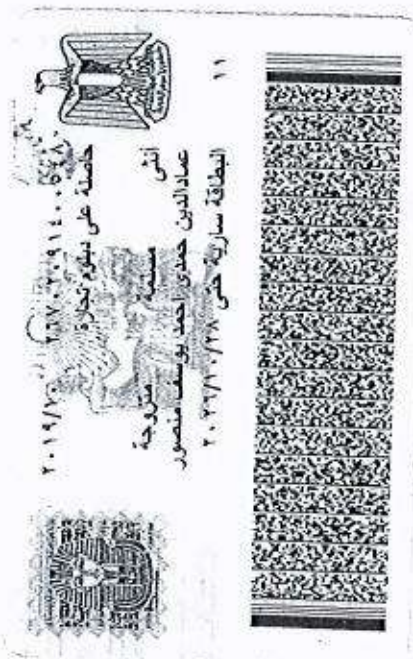
نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



PATIENT NAME: SAHAR ABD ELRAHMAN DARWISH ALI
GENDER: Female
DATE : 29-Nov-2025 09:56 AM
PATIENT ID: 2019112140
AGE: 58 y
ORDER: 3643650,3643651

POST- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

** Reviewing the previous study dated 27/07/2025 the current study shows:

1-CHEST

- Evidence of right mastectomy with still no sizable operative bed enhancing lesions.
- **Stable appearance of the previously noted:**
 1. Right upper and middle lung lobes show subpleural reticulations, likely post radiotherapy sequel.
 2. Prominent retrocaval, bilateral hilar and pre-vascular as well as left supraclavicular lymph nodes, the largest seen in the retrocaval group still measuring about 12 x 14 mm.
 3. Right middle subpleural nodule measuring about 6 mm.
 4. Left upper lobe anterior segment small pulmonary nodule still reaching 4 mm in diameter.
 5. Bilateral apical reticular infiltration likely representing old granulomatous.
- Left basal atelectatic band still seen.
- Otherwise, no pulmonary masses, areas of consolidation or cavity lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape. No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. Still No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the spleen, kidneys (apart from tiny left renal cortical cyst), pancreas, aorta, IVC and right suprarenal gland.

PATIENT: SAHAR ABD ELRAHMAN DARWISH ALI 2019112140, Female, 58 y
Report version 2, 30-Nov-2025 09:27 AM

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مركز باها البها
مركز باها البها
مركز باها البها

Name: سحر عبد الرحمن درويش على		
MR. : 4731		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 59-year-old postmenopausal individual with a history of invasive ductal carcinoma of the right breast, grade 2, estrogen-receptor score 8/8, progesterone-receptor score 6/8, HER2-negative, initially staged cT2N1. The patient underwent upfront right modified radical mastectomy with final pathology pT2N1, received adjuvant chemotherapy (doxorubicin-cyclophosphamide ×4 followed by docetaxel ×4), postoperative radiotherapy, and has been on tamoxifen since July 2020. In March 2022 bone metastases with lytic lesions were identified, prompting a switch to an aromatase inhibitor plus zoledronic acid. In April 2022 mediastinal and bilateral hilar lymph node progression was noted. Palliative radiotherapy was delivered to the left iliac bone and L2–L3 in August 2022. Disease progression on the aromatase inhibitor was documented in January 2024, after which a CDK4/6 inhibitor combined with letrozole was initiated in March 2024 and continues to be administered. The current regimen includes falodex and Ibrance. On 30 April 2026 a computed tomography scan demonstrated right-sided disease with additional findings described as “rd + sd + hfl (hemangioma ...)”; magnetic resonance imaging of the abdomen was pending. The patient was noted to be receiving Ibrance 125 mg on the day of assessment. On 24 May 2026 a dynamic MRI of the abdomen showed a transient hepatic attenuation difference (HFL). Laboratory results were within acceptable limits except for a creatinine of 1.2 mg/dL. The patient continues on Ibrance 125 mg.		
تحريراً في ,		
	التاريخ : 20-09-2026	الوقت :
	علياء احمد أبو النصر شهاب الدين	الطبيب المعالج :
يعتمد ,		

• Still noted:

1. Calcular gall bladder stone (measured 13 mm).
2. Bulky left adrenal gland.
3. Bilateral prominent external iliac lymph nodes.
4. Bulky uterus.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Small fat containing umbilical hernia is still seen.

The revised bone window revealed:

- **No gross interval changes regarding the previously noted:**
 1. Left posterior acetabular mixed lytic and sclerotic osseous lesion breaching the overlying cortical outlines with no associated sizable extra osseous soft tissue components.
 2. L3 posterior vertebral body small lytic lesion with marginal sclerosis measures about 15 x 18 mm in axial planes
 3. D10 mixed lytic/sclerotic transformation osseous lesion.

OPINION:

*****Status follow up shows:**

- Stationary course since the last study dated 27/07/2025 with no newly developed lesions.

Reported By,

Dr. Naglaa Mahmoud , Msc

Dr. GHADA KHALIFA, MD

30-Nov-2025 09:27 AM

PATIENT: SAHAR ABD ELRAHMAN DARWISH ALI, 2019112140, Female, 58 y
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مركز طبية البهية

مختبر طبية البهية

مختبر طبية البهية

مختبر طبية البهية



Patient Name: Sahar Abd Al Rahman Darwish	Date : 06/03/2025
Exam : C.T CHEST	Age : 58 Year(s)
Medical ID: 370/2024	Pat. ID: 47886

**NON- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

****A reported case of right breast cancer, ** As Compared
to the previous study dated 30/09/2024**

FINDINGS:

1. Chest:

- Right breast MRM with clear operative bed.
- **Stationary course as regards the following:**
 - Few bilateral scattered pulmonary nodules the largest is noted at the right upper lobe anterior segment pulmonary nodule, still measuring about 6mm.
 - Anterior right upper lobar sub-pleural fine reticulations, likely post-radiation sequel.
 - prominent sub-centimeteric mediastinal and hilar lymph nodes.
 - prominent few sub-centimeteric left supraclavicular lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions (on non- contrast basis) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Large hyperdense gall bladder stone is **still seen** measuring about 10x8mm.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.

Patient Name: Sahar Abd Al Rahman Darwish	Date : 06/03/2025
Exam : C.T CHEST	Age : 58 Year(s)
Medical ID: 370/2024	Pat. ID: 47886

- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys (apart from **still noted** left few hypodense cortical simple cysts) with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUCD is seen in place.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton showing rather stationary course regarding:

- Left posterior acetabular mixed lytic and sclerotic osseous lesion with no extra-osseous soft tissue component.
- L3 and D10 vertebral bodies lytic focal lesions with no vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

Rather stationary course since the last study.

Dr. Ahmed Elgazar

Dr. Walid Mohamed, M.D.

MDT FORM

3 / 9 / 2025 G

Name	عبد الرحمن درويش	MRN	٢٧٠٠٠٠٠
Age	58	ID	26702091400548
Menopausal status	Post	Phone	01212458404
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

MDT

Clinical Data

BX:	IDC	R	ps: 1.	L			
ER:	tv 8/6	PR:	+6/6	HER 2:	-	Ki67:	
Sonomammography:	postmenopausal L. Nodes						
Mets Workup:	July 2020 for TAM + 2 LN → 4 AC, 4 Tact TAM 8/2020 → 3/2022						
Recommendations:	Bow Mets + Med LN DE 1/2022 L 1/2024. DP. St. Faslodes, Ibrance 3/2024 CT: Mild increase Med LN 12x14 mm more sclerosis						
MDT OUTCOME & PLAN	27/7/2024 Op to 8x10 cm + inc in size of 010 muscles more sclerosis						

only mild increase + sclerosis → Continue faslodes, Ibrance

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatry

MRN: 2019112140
Patient Name: SAHAR ABD ELRAHMAN DARWISH ALI
D.O.B: 09/02/1967
IC No.: 26702091400548
Sex: Female
Location: External Clinic

SURGICAL PATHOLOGY

Specimen examination - (1 Site)

Specimen Id	2002160086	Collection Date	16/02/2020 11:06 AM
Accession No	SUR20-824	Received Date	16/02/2020 11:08 AM
Sample Type	Tissue	Doctor	Abd ALLAH Mohamed Gamil

Clinical Information

Rt. breast IDC grade II with lobular differentiation, on path no, 284/20

Gross Picture

Received formalin-fixed specimen, labeled with the patient's name, MRN and laterality:

Rt. modified radical mastectomy specimen 22.5x19.5 cm, attached to axillary tail 6x5x3 cm, covered by skin ellipse 19.5x13 cm, including grossly unremarkable nipple. Serial sectioning revealed firm white mass (2.5x2.3x2 cm) surrounded by rubbery area 7x6 cm. The mass is located at lower outer quadrant, 1.8 cm from skin surface and 4 cm from the least resected deep margin. Rest of breast tissue is rubbery and fatty. Axillary fat dissection revealed multiple Lymph nodes, the largest one is 4x3 cm.

Microscopic Picture

A case of mixed invasive duct and lobular carcinoma, grade II. No associated insitu carcinoma. No lymphovascular emboli. Stroma is desmoplastic with mild inflammatory cell reaction. Breast tissue shows fibroadenosis and fibrocystic disease. Nipple is free of Paget's disease. Deep margin is free. Axillary L nodes positive for macrometastasis [3/18] with capsular invasion but no perinodal fat infiltration.

Diagnostic Summary**Tumour type:**

mixed invasive duct and lobular carcinoma, grade II.

Specimen type:

MRM

Laterality

Right

Lesion Size**Invasive tumour size/s (mm):**

2.5x2.3 cm

Multiple Lesions

No

Histological grade**Mitosis score (1-3):**

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

2

Histological grade (1-3):

Grade II, 7/9

Peritumoural lymphovascular invasion

Patient Name:	SAHAR ABD ELRAHMAN DARWISH	Gender:	Female
MRN:	2019112140	Ordered By:	eman ebraheem abdelaziz ali
DOB:	09/02/1967	Age:	58 Y 7 M 2 D
		Order ID:	3537620

NM BONE SCAN

Procedure Date: 10/09/2025 09:41:00 AM

Report:

Report: 99mTc-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 4/11/2024, the current study revealed:-

Rather stationary course of the previously partially regressed left acetabular bone lesion and DV10 lesion. Still the CT detected LV2 lesion is not seen on scintigraphic basis.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

Follow up bone scan shows rather stationary course.

Approved by: Prof. Huda Fahy, Nuclear Medicine Consultant

Date: 10/09/2025 Time: 14:00

Reported and electronically endorsed by:

Huda Fahy Elsayed Abdulhelim

Reported on: 10/09/2025 02:04:00 PM

Patient Name: SAHAR ABD ELRAHMAN DARWISH	Gender: Female
MRN: 2019112140	Ordered By: NOURHAN ABDALLA MOHAMMED AFIFY ALI
DOB: 09/02/1967 Age: 58 Y 7 M 2 D	Order ID: 3410149

CT ABDOMEN AND PELVIS

Procedure Date: 27/07/2025 10:06:00 AM

Report:

Report:

NON- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

** Compared to the previous study dated 06/07/2024, the today's study showed:

1-CHEST

- Status post right mastectomy no sizable operative bed enhancing lesions.
- No appreciable changes regarding:
- 1. Right upper and middle lung lobes show subpleural reticulations, likely post radiotherapy sequel.
- 2. Right middle subpleural nodule measuring about 6 mm.
- 3. Left upper lobe anterior segment small pulmonary nodule reaching 4 mm in diameter.
- 4. Bilateral apical reticular infiltrations likely representing old granuloma
- Left basal atelectatic band.
- Otherwise, no sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Mild increase in size of the retrocaval lymph node, currently measuring about 12 x 14 mm compared to 8 x 10 mm previously.
- Stable appearance of the other smaller left supraclavicular, bilateral hilar and pre-vascular lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the spleen, kidneys (tiny left renal cortical cyst), pancreas, aorta, I.V.C and right suprarenal gland.
- Still noted unchanged:
- 1. Calculus gall bladder stone (measured 13 mm).
- 2. Bulky left adrenal gland.
- 3. Bulky uterus.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

PATIENT NAME: SAHAR ABD ELRAHMAN DARWISH ALI
GENDER: Female
DATE : 31-Aug-2026 10:16 AM

PATIENT ID: 2019112140
AGE: 59 y
ORDER: 3973490,3973491

POST- CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

** reviewing previous MRI study dated 24/05/2026 was reviewed;
** Reviewing the previous CT study dated 30/04/2026 the current study shows:

1-CHEST

- Right mastectomy with still no sizable operative bed enhancing lesions, or collections.
- progression regarding the enlarged left supradivicular lymph nodes largest measures 14 x 10 mm (compared to 12 x 8 mm in the last study)
- Mild size progression of the prominent mediastinal lymph nodes, the largest is paratracheal measures about 13 x 10 mm compared to 10 x 8 mm previously.
- Stable appearance of the previously noted:
 1. Right upper and middle lung lobes show subpleural reticulations, likely post radiotherapy sequel.
 2. Right middle subpleural nodule measuring about 6 mm.
 3. Left upper lobe anterior segment small pulmonary nodule still reaching 4 mm in diameter.
 4. Bilateral apical reticular infiltration likely representing old granulomatous.
- Left basal atelectatic band still seen.
- Otherwise, no pulmonary masses, areas of consolidation or cavitory lesions.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape. No pericardial effusion detected.

2- ABDOMEN AND PELVIS

- The liver is of average size with homogenous CT texture. Currently

PATIENT: SAHAR ABD ELRAHMAN DARWISH ALI, 2019112140, Female, 59 y

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non visualized right lobe segment VII lesion was enhancing in previous CT (corresponding to previous MRI dated 24/05/2026) showing no diffusion restriction or contrast enhancement, likely representing Transient hepatic attenuation difference.

- **Currently appreciated** hypodense non enhancing focal lesion at site of falciform ligament measuring about 14x23 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal appearance of the spleen, kidneys (apart from tiny left renal cortical cyst), pancreas, aorta and IVC.
- **Currently noted:**
 1. Mild to moderate right pelvic calyceal system and proximal two thirds of ureter dilatation with no obstructing stone ? Passed stone, associated with right peri-nephric fat smudging.
 2. More appreciated thickening of medial perihepatic peritoneal reflection/ right diaphragmatic crus along right para-vertebral region seen hardly separable from right bulky adrenal gland noted at medial aspect of right kidney. ?soft tissue sheets.
- Still noted:
 1. Calculus gall bladder. Focal mural thickening seen at its fundus, likely small polyp.
 2. Bulky left adrenal gland.
 3. Bilateral prominent external iliac lymph nodes.
 4. Bulky uterus showing hypodense distention of its cavity for ultrasound assessment.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Small fat containing umbilical hernia.

he revised bone window revealed:

- **Increase lytic burden of :**
 1. left posterior acetabular mixed lytic and sclerotic osseous lesion breaching the overlying cortical outlines with no associated sizable extra osseus soft tissue components.
 2. D10 mixed lytic/sclerotic transformation osseous lesion with mild central wedging.
- **Currently noted osteopenic texture of examined bones showing widespread scattered multi-level vertebral lucencies for MR assessment if clinically justified.**
- **No gross interval changes regarding the previously noted:**
 1. Dorsolumbar spondylosis.
 2. L3 posterior vertebral body small lytic lesion with marginal sclerosis.
 3. First degree degenerative spondylolisthesis of L5.

Status follow up shows:

- Still no sizable operative bed masses or collections.
- Mild size progression of the size of :
 1. The mediastinal lymph nodes.
 2. Left supraclavicular lymph node
- Currently noted:
 1. soft tissue thickening of the right diaphragmatic crus & the medial perihaptic peritoneal reflection with no clear separation could be noted from the right bulky adrenal gland , likely metastatic
 2. Osteopenic texture of examined bones showing widespread scattered multi-level vertebral lucencies for MR assessment .
 - Increase lytic burden of:
 1. Left posterior acetabular mixed lytic and sclerotic osseous lesion breaching the overlying cortical outlines with no associated sizable extra osseus soft tissue components.
 2. D10 mixed lytic/sclerotic transformation osseous lesion with mild central wedging.
 - Currently non visualized right lobe segment transient hepatic attenuation difference.
 - Currently noted hypodense non enhancing focal lesion at site of falciform ligament, likely focal fatty infiltration/pseudo-lesion.
 - Stationary course regarding:
 1. Few pulmonary nodules as described.
 2. Bulky uterus showing hypodense distention of its cavity for ultrasound assessment.
 3. L3 vertebral lesion as described.

Reported By,
 Dr. Shaimaa Khaled, MSc, FEBR
 Dr. Hekmat Samir, MD 01-Sep-2026 03:05 PM

PATIENT NAME: SAHAR ABD ELRAHMAN DARWISH ALI

GENDER: Female

DATE : 09-Aug-2026 10:34 AM

PATIENT ID: 2019112140

AGE: 59 y

ORDER: 3973391

99MTC-MDP BONE SCAN**Desired Information: -**
skeletal survey.**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:**Compared to previous bone scan dated 10/9/2025, the current study revealed:-**

Rather stationary course of the previously seen left acetabular bone lesion and DV10 lesion with currently seen more sclerosis that is likely due to therapy effect rather than progression.

Still the CT detected LV2 lesion is not seen on scintigraphic basis.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:**More or less stationary course.****Reported By,****Dr. Huda Fathy, MD 09-Aug-2026 01:42 PM**

PATIENT: SAHAR ABD ELRAHMAN DARWISH ALI / 2019112140 / FEMALE, 59 y

ACCESSION: 166802

EXAM: 99mTc-MDP, 2019112140

PATIENT ID: 2019112140

AGE: 59 y

ORDER: 3973391

PATIENT: SAHAR ABD ELRAHMAN DARWISH ALI / 2019112140 / Female, 59 y

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مركز بهية
4 شارع عطية ونور شارع البرد جسر طاري، الجيزة
مستشفى بهية الشيخ زايد
1- د. هادي فathy - الشيخ زايد رقم كنية 166802**Technique:**

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

Compared to previous bone scan dated 10/9/2025, my current study revealed: Rather stationary course of the previously seen left acetabular bone lesion and DV10 lesion with currently seen more sclerosis that is likely due to therapy effect rather than progression.

Still the CT detected LV2 lesion is not seen on scintigraphic basis.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:**More or less stationary course.****Reported By,****Dr. Huda Fathy, MD 09-Aug-2026 01:42 PM**